OMEGA CARE

Behaviour Support Plan

PRICE

Protecting Rights in a Caring environment

Name of young person:	
Plan created by:	
Date of plan initially created:	
Last Reviewed:	
Author:	

Use of Restraint Regulation 20 sets out the only purposes for which restraint can be used:

1. Preventing injury to any person (including the child who is being restrained)
2. Preventing serious damage to the property of any person (including the child who is being restrained)
3. Preventing a child who is accommodated in a secure children's house from absconding from the house.

When restraint involves the use of force, the force used must not be more than is necessary and should be applied in a way that is proportionate i.e., the minimum amount of force necessary to avert injury or serious damage to property for the shortest possible time. Restraint that deliberately inflicts pain cannot be proportionate and should never be used on children in children's houses.

There may be circumstances where a child can be prevented from leaving a house – for example a child who is putting themselves at risk of injury by leaving the house to carry out gang related activities, use drugs or to meet someone who is sexually exploiting them or intends to do so. Any such measure of restraint must be reasonable, proportionate and in place for no longer than is necessary to manage the immediate risk, it should be the last resort.

In a restraint situation, the team should use their professional judgement, supported by their knowledge of each child's risk assessment, an understanding of the needs of the child (as set out in their relevant plans) and an understanding of the risks the child faces. Professional judgements may need to be taken quickly, and the team training, and supervision of practice should support this.

Any use of restraint carries risks. These include causing physical injury, psychological trauma, or emotional disturbance. When considering whether restraint is warranted, the team in children's houses need to consider:

1. The age and understanding of the child.
2. The size of the child.
3. The relevance of any disability, health problem or medication to the behaviour in question and the action that might be taken as a result.
4. The relative risks of not intervening.
5. The child's previously sought views on strategies that they considered might deescalate or calm a situation, if appropriate.
6. The method of restraint which would be appropriate in the specific circumstances.
7. The impact of the restraint on the carer's future relationship with the child.

views on intervention.
Specific/health needs to be considered relating to behaviour
baseline behaviours
Triggers
Environmental triggers (smell, noises, certain actions, other peers behaviour) • Internal actions (anxiety, trauma, thoughts) •

Stage 1 (Anxieties/Triggers that may cause the individual to show low level disruptive behaviour or display small signals that their behaviour may escalate)	
•	
Strategies and responses	
Helpful	Unhelpful
•	•
Views/opinions	views/opinions

Stage 2 (Conflict – At this stage, the individual may show an increase in their behaviour causing a conflict as the behaviour may challenge others)
•

Stage 3 (Crisis – At this stage, the individual may become physically aggressive towards other people try to significantly damage property or try to harm himself and a physical intervention may be required to keep them and others around them safe)

If all de-escalation attempts have failed and Owen's behaviour continues to present a risk to self or others the team may need to consider using PRICE holds as a last resort which would be proportionate, necessary, and lawful for the least amount of time. Physical interventions are used if:

- Risk to self or others.
- Substantial property Damage with a consequence of harm.
- Endangerment of life/ Health

If the team have exhausted all options at their disposal they can try and nothing has worked, it is at this point they would use physical intervention. Holds are to be practiced as a last resort if it is their honestly held belief that the risk of not intervening is likely to cause greater harm to keep Owens and others safe and would only be used for the least amount of time needed.

Owen understands that the team will be using intervention holds to help keep him and others safe and that medical attention must be offered afterwards if he needs it.

- All team members to be trained in the use of PRICE restrictive physical interventions.
- There are no medical concerns that would contraindicate restraint (prohibit).
- Young person's dignity should be always maintained.
- If other young people are present, then they should be asked to remove themselves from the area.

PRICE techniques to be used

- Single embrace
- Single adapted Embrace

Strategies and responses	
Helpful	Unhelpful
•	•
Young person - Views/opinions	Young person - Views/opinions

Strategies and responses	
Helpful	Unhelpful
•	
Young person - Views/opinions	Young person - views/opinions

Stage 4 (Recovery- The individual needs support to recover and calm from stage 3 crisis behaviour)

•

Strategies and responses	
Helpful	Unhelpful
•	
Young person - Views/opinions	Young person - Views/opinions

Authors Name: Joanna Whiting

Authors Signature:

I agree that I have read and understood the above risk management plan. I understand the protocols and control measures and I am / we are aware of my / our responsibilities in meeting these actions.

Name	Date	Relationship to the young person	Signature